

العمر(تاريخ الميلاد) : 07-11-1987 المهنة : ربه منزل

التوقيع: (_____)

التعليق على التشخيص : -----

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

رئيس
المجلس الطبي المتخصص

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Name: نورا مسعد عبد الفتاح عبد الحليم National ID: 28711072101609	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نورا
مساعد عبد الفتاح عبد الحليم
قرية الشوبك الشرقي
مركز الصف - الجيزة

١٩٨٧/١٢/٢٧ ٢٨٧ ١١ ٠٧ ٢١ ٠١٦ ٠٩
JY4116530

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تعاليل وامشاع
رئيس

٢٠٢٣/٠١ ٢٨٧ ١١ ٠٧ ٢١ ٠١٦ ٠٩

رؤية منزل
أنثى
مستلمة
مستلمة
علاء الدين يوسف خليل سيد احمد
البطاقة سارية حتى ٢٠٣٠/٠١/٢٧

١

Name : Nora Mosaad Abdelfatah
Branch : Portable
Ref.Physician Prof : CAP+C

Date : 26-02-2026 09:47
Medical No. : GRI-98863
Ins.Company :

**MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH
IV CONTRAST:**

- Compared to pervious limited CT REPORT DATED 23-9-2025, CURRENT STUDY REVEALED:

I Chest:

- Left breast showed diffuse skin thickening with no sizable masses.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- **Gall bladder** multiple intraluminal small stones *for US assessment*, No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- L1 lytic lesion is noted measuring 1.2 X 1.6 cm with cortical breaching and subtle posterior collapse.

Opinion:

- Stationary course since the last study.

Dr. Asser Ahmed

Name : Nora Mosaad Abdel Fattah
Branch : Dokki
Ref.Physician Prof : Gostaf Rosch Hospital

Date : 27-11-2025 10:45
Medical No. : 259951
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 38-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Increased tracer uptake at LV1.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Active tracer uptake at LV1, for more radiological correlation.
- The rest of skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name: Nora Mossad Abd Al Fattah	Date: 23/09/2025
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Age: 38 Year(s)
Medical ID: 980/2025	Pat ID: 98863

MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH IV CONTRAST:

- Reported case of breast cancer.
- Compared to previous limited CT REPORT DATED 10/4/2025, CURRENT STUDY REVEALED:

I Chest:

- Left breast showed diffuse skin thickening with no sizable masses.
- Enlarged left axillary lymph nodes, largest averaging 1.3 x 2 cm. **... for sonomamographic assessment.**
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- **Gall bladder hydrops** averaging 13x4 cm with multiple intraluminal small stones **for US assessment**, No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- L1 lytic lesion is noted measuring 1.2 X 1.6 cm with cortical breaching and subtle posterior collapse.

Opinion:

Rather stationary course:

- L1 lytic lesion ?metastatic...for bone scan assessment.
- No sizable pulmonary or hepatic deposits on non-contrast basis.

Dr. Reda Mabrouk, MD

Patient Name : الأستاذة / نورا مسعد عبد الفتاح عبد الحليم	Request No. : 13003135
Patient No. : 28711072101609	Request Date : 17/02/2025
Path No. : 694/25	Verification Date : 05/03/2025
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 37 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left UIQ breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 16/2/2025, 12:20 pm.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 60% of tumor cells (score 6/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 35% nuclear reaction of tumor cells.

Diagnosis:

Left UIQ breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 6 /8).
- PR: Positive (score 8 /8).

Verified by :

Dr.Toka Medhat

Printing Date : 24/03/2025 11:34:27

Page 1 of 2

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CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / نورا مسعد عبد الفتاح عبد الحليم	Request No.	: 13003135
Patient No.	: 28711072101609	Request Date	: 17/02/2025
Path No.	: 694/25	Verification Date	: 05/03/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Her2: Positive (score 3+).
- Ki67: Positive in 35% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong) Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

Dr.Toka Medhat

Printing Date : 24/03/2025 11:34:27

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Nora Mossad Abd Al Fattah	Date : 23/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 38 Year(s)
Medical ID: 980/2025	Pat. ID: 98863

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- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- L1 lytic lesion is noted measuring 1.2 X 1.6 cm with cortical breaching and subtle posterior collapse.

Opinion:

Rather stationary course:

- L1 lytic lesion ?metastatic...for bone scan assessment.
- No sizable pulmonary or hepatic deposits on non-contrast basis.

Dr. Reda Mabrouk, MD



Patient Name: Nora Mossad Abd Al Fattah	Date : 10/04/2025
Exam : C.T CHEST	Age : 37 Year(s)
Medical ID: 980/2025	Pat. ID: 98863

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Left breast UIQ irregular lesion inseparable from the underlying pectoral muscle, it measures 1.8 X 3.6 cm. diffuse left skin thickening.
- Enlarged left axillary lymph nodes at level I and II reaching 1.3 x 2 cm.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- L1 lytic lesion is noted measuring 1.2 X 1.6 cm with cortical breaching and subtle posterior collapse.

Opinion:

- Reported case of breast cancer revealed:
- L1 destructive lytic lesion likely metastatic.
- No sizable pulmonary or hepatic deposits on non-contrast basis.

Dr. Mohamed Mohsen, M.D.

□ Name: Noura Mosaad Abdul Fattah

□ Date : 12-Feb-2025

BILATERAL BREAST SONO-MAMMOGRAPHY REVEALED:

- Average left breast density (ACR A).
- Left ill defined multi-focal hyperdense mass lesion seen with micro-calcification seen at left UIQ.
- Relatively increased skin thickness.

Complementary US revealed:

- The clinically felt swelling in left UIQ opposite 10-11 o'clock revealed rather-defined irregular outline hypoechoic heterogeneous bi-focal soft tissue mass lesion with surrounding architecture distortion and dismoblastic reaction. Color doppler revealed minimal vascularity, the largest measuring about (34x19x26 mm)
- Another smaller similar lesions seen at the same quadrant at the most lateral aspect measuring about 20x14 and 12x10 mm.
- Ipsilateral suspicious axillary lymphadenopathy with distorted hila, the largest measuring 22x17 mm.
- No ductectasia.
- Normal skin thickness and contour
- Normal right breast and axillae.

CONCLUSION:

LEFT UIQ MULTI-CENTERIC SUSPICIOUS SOFT TISSUE MASS LESIONS
WITH IPSILATERAL SUSPICIOUS NODAL AXILLARY
LYMPHADENOPATHY AS DESCRIBED... CATEGORIZED AS (BIRADS V)
FOR HISTO-PATHOLOGICAL ASSESSMENT

Dr. Ahmed Taha. MSc



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Nora Mosaad Abdel Fattah Abdelhalim
Branch : Dokki
Ref.Physician Prof : Goustav Rousey Egypt

Date : 13-06-2026 08:41
Medical No. : 392332

Tc-99m MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

38 year-old female patient, who gave history of left breast cancer, received chemo-radiotherapy and hormonal treatment complains of bony ach and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- *Considering the last bone scan report dated 27/11/2025*, the current scan revealed uneven radiotracer uptake opposite the left shoulder posteriorly.
- The previously noted lesion at L V1 is not seen on the current study.
- Degenerative changes are noted at different levels of the spine and big joints.
- Otherwise rest of the scanned skeleton is unremarkable.

IMPRESSION:

- Uneven radiotracer uptake opposite the left shoulder posteriorly, for CT correlation.
- Unremarkable rest of the surveyed skeleton.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

N I L E S C A N & L A B S C E N T E R